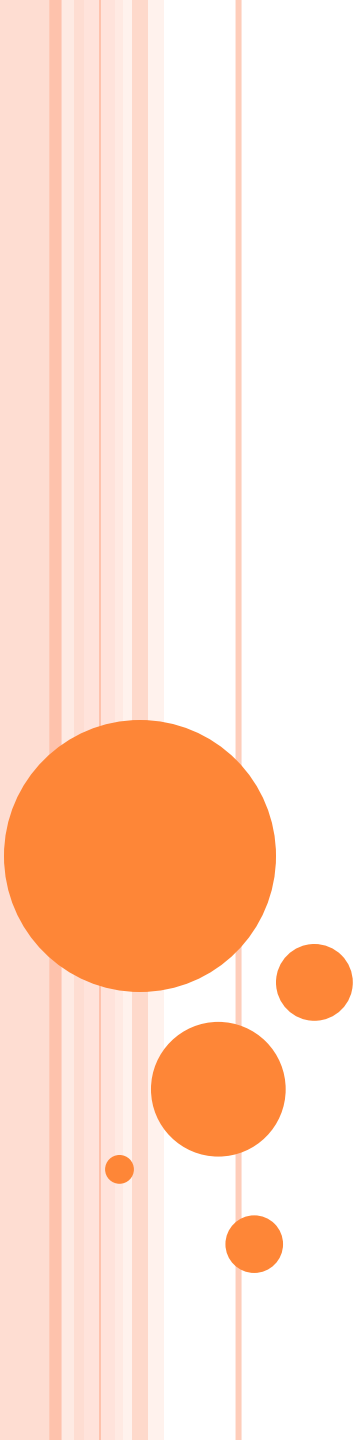


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LOCOMOTORS TRAINING IS A TYPE OF THERAPY THAT CAN HELP PEOPLE WITH SPINAL CORD INJURY IMPROVE OR RECOVER THEIR ABILITY TO WALK. THIS IS DONE THROUGH REPETITIVE PRACTICE AND WEIGHT-BEARING ACTIVITIES.

LOCOMOTOR TRAINING CAN INCLUDE A VARIETY OF DEVICES AND TECHNIQUES. THESE CAN VARY DEPENDING ON THE FACILITY THAT OFFERS THE THERAPY. LOCOMOTOR TRAINING MAY BE DONE ON OR OFF A TREADMILL WITH BODY-WEIGHT SUPPORT. SOMETIMES A ROBOT-ASSISTED BODY-WEIGHT-SUPPORT TREADMILL SYSTEM IS USED.



TRIGEMINAL NEURALGIA

- Trigeminal neuralgia operations are reserved for patients whose debilitating, shock-like facial pain cannot be managed with medication. The three primary surgical approaches are **microvascular decompression** (correcting nerve compression), **stereotactic radiosurgery** (using radiation), and **percutaneous procedures** (blocking nerve signals via needles)





Endocrine diseases are caused by **hyposecretion** (gland produces too little hormone) or **hypersecretion** (too much hormone). These imbalances directly affect metabolism, growth, and bodily functions.

1. Pituitary Gland

Growth Hormone (GH):

Hyposecretion: **Pituitary Dwarfism** (impaired skeletal growth in children).

Hypersecretion: **Gigantism** (in children) or **Acromegaly** (enlarged hands/feet in adults).

Hyposecretion: **Diabetes Insipidus** (excessive thirst and urination).

2. Thyroid Gland

Thyroid Hormones

Hyposecretion: **Hypothyroidism** (causes fatigue, weight gain, and cold intolerance).



3. Adrenal Glands

Cortisol:

Hyposecretion: Addison's Disease (low blood pressure, extreme fatigue, skin darkening).

Hypersecretion: Cushing's Syndrome (weight gain in the face/neck, high blood pressure, skin striations).

4. Pancreas

Insulin:

Hyposecretion: Diabetes Mellitus Type 1 (inability to regulate blood sugar, causing hyperglycemia).

Hypersecretion: Hyperinsulinism / Hypoglycemia (abnormally low blood sugar).

5. Parathyroid Glands

Parathyroid Hormone (PTH):

Hyposecretion: Hypoparathyroidism (low blood calcium causing muscle cramps/tetany).

Hypersecretion: Hyperparathyroidism (weak bones, high blood calcium, kidney stones)

REPRODUCTIVE DISORDERS

PCOS is a hormonal disorder that affects women of reproductive age. It's characterized by the presence of multiple small cysts on the ovaries (hence "polycystic"), irregular menstrual cycles, and an overproduction of male hormones (androgens). These hormonal imbalances can lead to a wide range of symptoms that affect both your physical and emotional well-being.

PCOD, or **Polycystic Ovarian Disease**, is a condition where the ovaries release immature or partially matured eggs that eventually turn into cysts. Over time, this can cause the ovaries to enlarge, leading to a hormonal imbalance similar to PCOS. The main difference is that PCOD is less severe and often easier to manage than PCOS. While the symptoms of both conditions overlap, **PCOS** is a more complex syndrome that can have long-term implications on your metabolic health, and even your heart.



What Causes PCOS and PCOD?

The exact **causes of polycystic ovaries** aren't fully understood, but several factors seem to contribute to these conditions. Here are some potential causes:

1. Hormonal Imbalance

The main issue in both **PCOS** and **PCOD** is an imbalance of reproductive hormones. Women with these conditions tend to have higher levels of androgens (male hormones), which can interfere with the development and release of eggs during the menstrual cycle. This can lead to the formation of **cystic ovaries**.

2. Genetics

If your mother, sister, or other female relatives have **PCOS** or **PCOD**, there's a higher chance that you may develop it too. Research shows that genetics play a significant role in the development of these conditions.



3. Insulin Resistance

Insulin resistance is one of the leading causes of **PCOS**. When the body becomes resistant to insulin, it produces more to compensate. High insulin levels stimulate the ovaries to produce more androgens, contributing to **bilateral polycystic ovaries** and other symptoms like weight gain and acne.

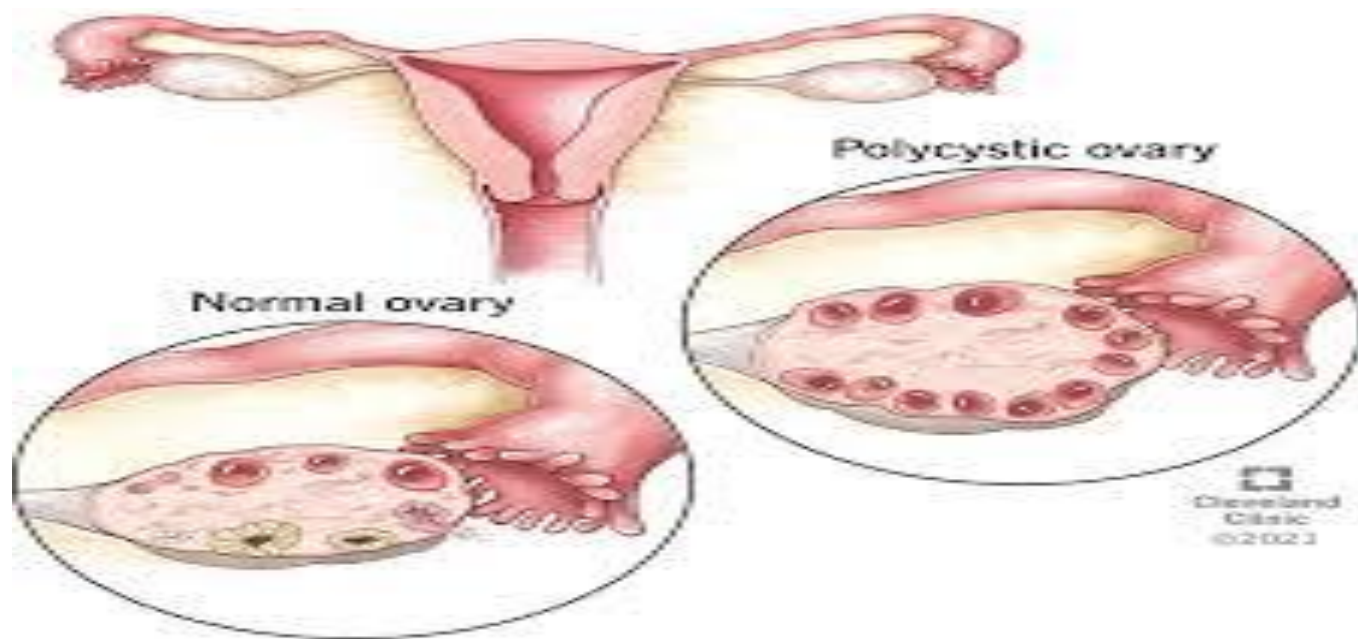
4. Inflammation

Women with **PCOS** often have low-grade inflammation, which can prompt the ovaries to produce androgens. Chronic inflammation is linked to several health issues, including heart disease, another risk factor associated with **polycystic ovary syndrome**.

5. Lifestyle Factors

Diet and physical activity can also influence the development of **PCOS** or **PCOD**. A poor diet high in processed foods and a sedentary lifestyle can exacerbate insulin resistance, leading to more severe symptoms.

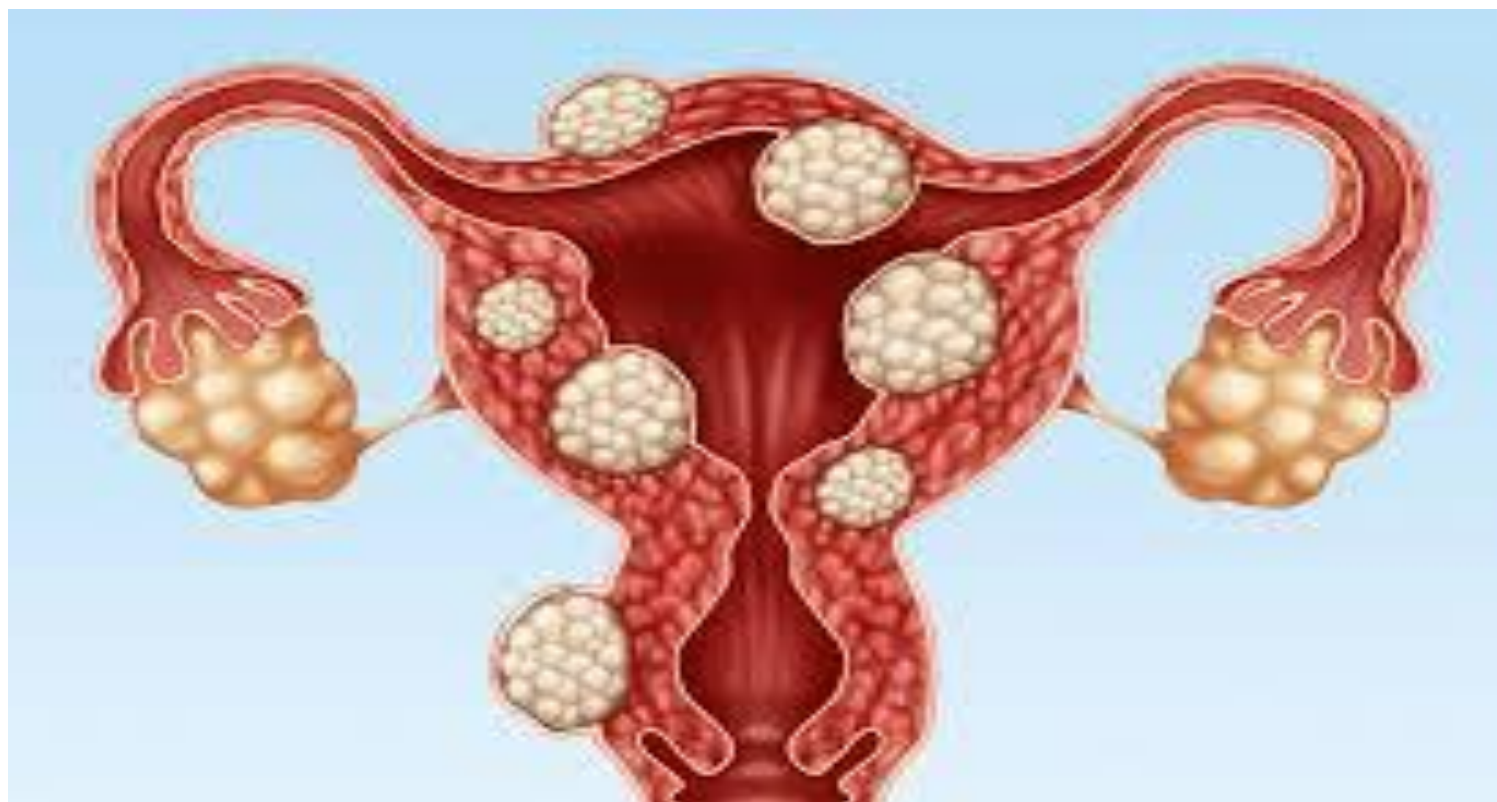




UTERINE FIBROIDS

Uterine fibroids (leiomyomas) are noncancerous growths in the muscular wall of the uterus. While some people experience no symptoms, others suffer from heavy menstrual bleeding, pelvic pain, and pressure. Fibroids are very common, typically managed through medication, minimally invasive procedures, or surgical removal





SYMPTOMS:

Many people who have uterine fibroids don't have any symptoms. In those who do, symptoms can be influenced by the location, size and number of fibroids.

The most common symptoms of uterine fibroids include:

Heavy menstrual bleeding or painful periods.

Longer or more frequent periods.

Pelvic pressure or pain.

Frequent urination or trouble urinating.

Growing stomach area.

Constipation.



Epididymitis (ep-ih-did-uh-MY-tis) is an inflammation of the coiled tube, called the epididymis, at the back of the testicle. The epididymis stores and carries sperm. Males of any age can get epididymiti

Epididymitis that lasts longer than six weeks or that happens over and over again is considered chronic. Symptoms of chronic epididymitis might come on slowly. Sometimes the cause of chronic epididymitis isn't able to be found.



Epididymitis

